

**Public Health Emergency Operations Center
"COUSP DRC"**

MVE-17 Incident Management System

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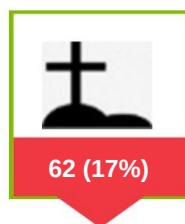
Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°019/MVB_02/06/2026

Provinces affected (3)	ITURI, NORTH KIVU & SOUTH KIVU
Affected Health Zones (25)	ITURI : Aru, Aungba, Bambu, Damas, Bunia, Gety, Kilo, Komanda, Lita, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rimba , Rwampara and Logo NORTH KIVU : Butembo, Goma, Oicha, Kalunguta, Katwa, Kyondo and Beni SOUTH KIVU : Miti-Murhesa
Reporting date	June 2, 2026
Publication date	June 3, 2026



Total confirmed cases
for the 3 provinces



Cumulative deaths
among confirmed
cases and case fatality rate



Patients in
isolation -
hospitalization



Cumulative number
of recoveries



Follow-up rate of
contacts for the 3
provinces

*Data currently being harmonized. **Location of these confirmed active cases is underway in order to harmonize statistics.

1. HIGHLIGHTS

- **Nineteen (19) new confirmed cases, including two deaths**, were notified on June 2, 2026. These cases are located in the health zones of **Rwampara (7), Bunia (5), Nyankunde (3), Rimba (3) and Damas (1)**.
- **The Rimba health zone (Ituri) is newly affected**, bringing the number of affected zones to 17 in this province.
- **In North Kivu, teams are preparing for the release of the first recovered case in Goma on June 3, 2026.**
- A serious incident occurred in South Kivu, where Safe and Dignified Burial (SDB) teams were attacked by residents in Katana. The coffin was abandoned and the body handled by the community, creating a major risk of new chains of transmission.
- **Joint mission (DG INSP, IM WHO, PF UNICEF and WFP) to evaluate Ebola response activities in the Mongbwalu health zone**, followed by discussions with political and administrative authorities and community leaders on their roles in prevention, early detection of cases and compliance with barrier measures.
- **Thirty-two (32) contacts in the Rwampara health zone successfully completed their follow-up 21 days.**

2. EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

Ituri province, located in the northeast of the Democratic Republic of Congo, shares a long and porous border with Uganda and South Sudan. For more than two decades, it has faced a chronic humanitarian crisis linked to armed conflict and recurring population displacements. Its population is estimated at over 8 million, including more than one million internally displaced persons. The Mongbwalu health zone, considered the starting point of the epidemic, is located in the Djugu territory, 70 km from Bunia. This area is characterized by the presence of armed groups and armed movements.

Frequent population movements towards Uganda and numerous artisanal mining sites attract migrant workers. In September 2018, Ituri province had already been affected by the 10th Ebola virus (EV) outbreak, which was then raging in North Kivu. The current outbreak, caused by Bundibugyo ebolavirus, was officially declared on May 15, 2026, by the Minister of Public Health. The three affected provinces have a total estimated population of nearly 15 million, with massive internal displacement and cross-border flows to neighboring countries.

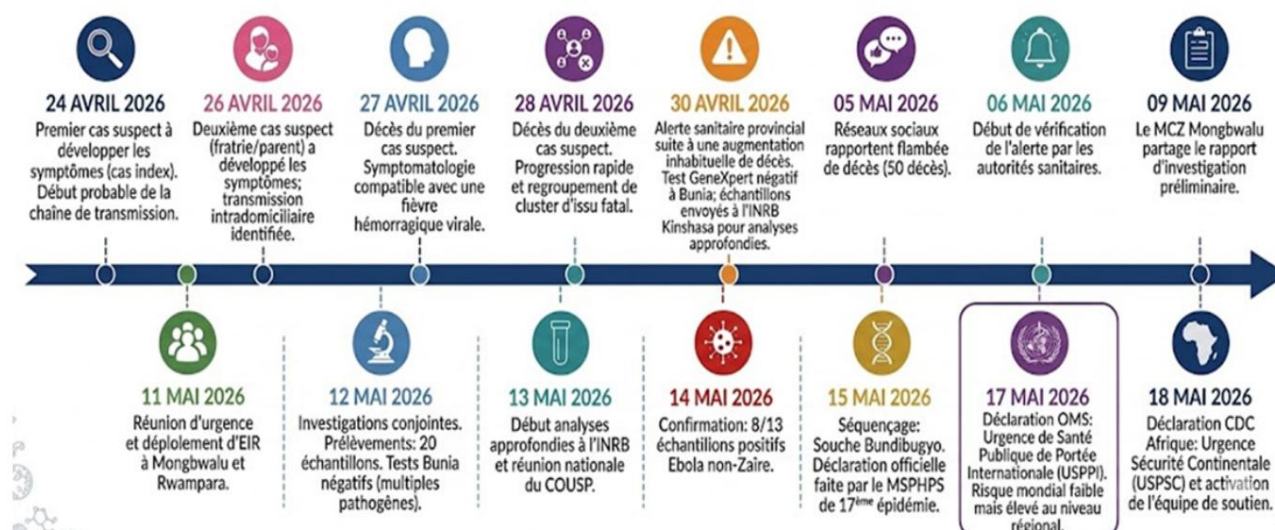


Figure 1. Chronology of events of the Ebola-Bundibugyo virus disease epidemic.

3. DETAILED EPIDEMIOLOGICAL ANALYSIS

3.1 Spatial distribution and active hotspots

Province	Confirmed cases	Deaths (confirmed)	Case fatality rate	Affected health zones	New cases
Ituri	341	48	14.1% 17/36	68.4%	19
North Kivu	19	13	7/34	33.3% 1/34	0
South Kivu	3	1	17.1%	25/104	0
TOTAL	363	62			19

- Ituri remains the national epicenter with 93.9% of confirmed cases. Although the case fatality rate is lower there (14.1%), the geographical expansion towards Rimba indicates active community transmission.
- North Kivu has a worrying case fatality rate of 68.4%, explained by delays in treatment and the escape of three confirmed cases from healthcare facilities.

3.2 Distribution by health zone (Cumulative as of 02/06/2026)

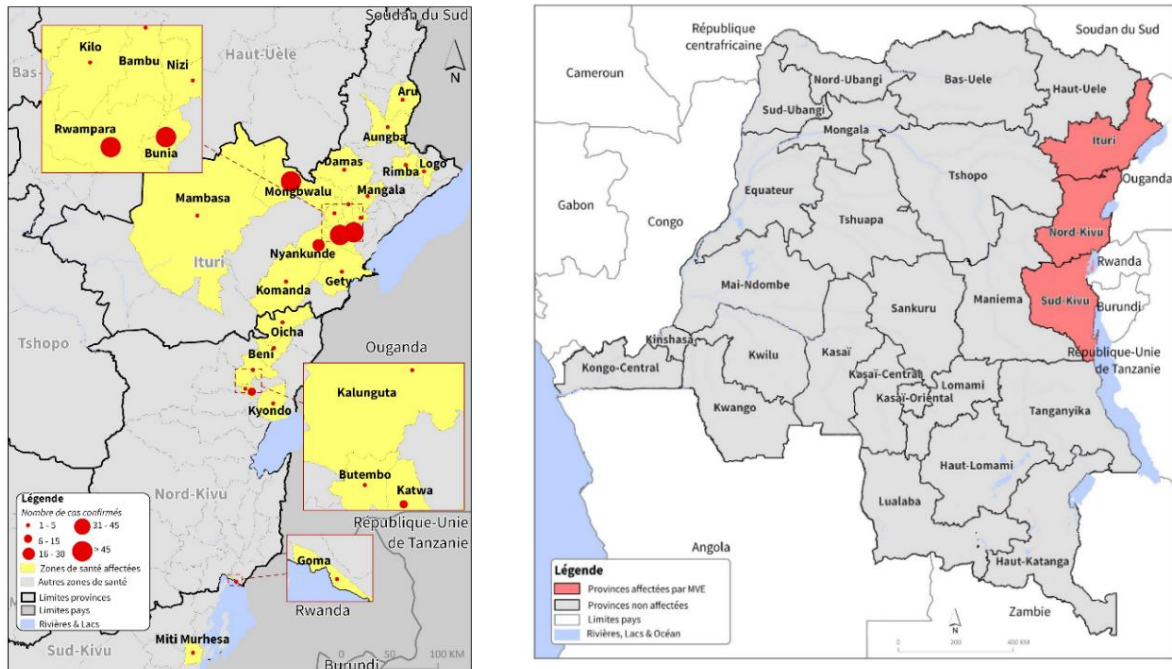
- **Ituri Province (341 cases):** Bunia (85), Rwampara (72), Mongbwalu (47), Nyankunde (22), Rimba (3), Mambasa (2), Bambu (2), Aru (2), Damas (2), Kilo (2), Nizi (2), Mangala (1), Aungba (1), Gety (1), Komanda (1), Lita (1), Logo (1). Ninety-four (94) additional cases are being ventilated.
- **North Kivu Province (19 cases):** Katwa (7), Beni (5), Butembo (2), Oicha (2), Kalunguta (1), Kyondo (1), Goma (1).
- **South Kivu Province (3 cases):** Miti-Murhesa (3).

3.3 Demographic analysis: The **18-49** age group is the most affected, with a **male predominance among confirmed cases** (approximately 55% male).

Table 1. Distribution of confirmed cases and deaths of Bundibugyo Ebola virus disease by health zone and province, Democratic Republic of Congo, as of June 2, 2026

Province / Health Zone	Cumulative confirmed cases (n)	Cumulative confirmed deaths (n)	Lethality (CFR, %)
ITURI			
1. Bunia	85		9.4
2. Rwampara	72	8 15	20.8
3. Mongbwalu	47	10	21.3
4. Nyankunde	22	1	4.5
5. Bambu	2	2	100.0
6. Aru	2	1	50.0
7. Kilo 8.	2	0	0.0
Nizi	2	0	0.0
9. Mangala	1	0	0.0
10.	2	0	0.0
Damascus	1	0	0.0
11.	1	0	0.0
Aungba 12.		0	0.0
Gety 13. Komanda 14. Lita	11	0	0.0
15. Logo	1	0	0.0
16. Mambasa	2		50.0
17. Rimba	3	10	0.0
- Other ZS (undisclosed data)	94	10	10.6
Ituri subtotal	341	48	14.1
NORTH KIVU			
1. Katwa 2.	7	5	71.4
Beni	5	3	60.0
3. Butembo	2	2	100.0
4. Oicha	2	2	100.0
5. Kalunguta	1	1	100.0
6. Kyondo	1	0	0.0
7. Goma			0.0
North Kivu subtotal	1 19	0 13	68.4
SOUTH KIVU			
1. Miti-Murhesa	3	1	33.3
South Kivu subtotal	3	1	33.3
TOTAL	363	62	17.1

3.4 Epidemic mapping



Figures 2 & 3. Spatial distribution of confirmed Ebola cases by affected health zone and in the three affected provinces as of June 2, 2026

4. RESPONSE ACTIONS BY PILLAR

4.1 Coordination

- **National / Kinshasa:** Tripartite meeting between the Ministry of Health, Hygiene and Social Welfare (through COUSP/INSP, SGI), the Ministry of Higher and University Education, Scientific Research and Innovations as well as the Ministry of National Education and New Citizenship on the integration into the preparedness and response plan for the 17th epidemic of response activities in the School, University and Academic environment.
- **National / Ituri:** Joint mission (INSP, WHO, UNICEF, WFP) to Mongbwalu to assess the response and mobilize community leaders. Start of work on developing the consolidated operational plan (CONOPS).
- **North Kivu:** Sharing of the provincial operational plan and monitoring of travelers in quarantine. Coordination meetings to address the security challenge related to ADF incursions.
- **South Kivu:** Meeting with the Provincial Governor; recommendation for direct funding of activities to motivate service providers. Health Cluster meeting held to discuss mobility challenges.

4.2 Epidemiological surveillance

Table 2. Management of epidemiological alerts (24h) as of June 2, 2026

Indicator	Ituri	North Kivu	South Kivu	Total
Alerts raised	158	153	5	316
Alerts investigated	157	152	4	313
Investigation rate	99.4%	99.3%	80.0%	99.1%
Alerts confirmed	122	20	2	144

Table 3. Contact tracing of confirmed cases as of June 2, 2026

Province	Contacts under follow-up (n)	Contacts viewed (24h)	Follow-up rate (%)
Ituri	3,637	1,498	41.2
North Kivu	519	326	62.8
South Kivu	137	131	95.6
Total	4,293	1955	45.5

- **Challenges:** Persistent low performance in Ituri (41.2%) and North Kivu (62.8%). The escape of three confirmed cases in North Kivu and the attacks on EDS teams in South Kivu (Katana) and Ituri (Bunia) constitute major obstacles.

4.3 Laboratory

- **Ituri:** 70 samples collected and analyzed (positivity 27.1%); no samples pending.
- **North Kivu:** 34 samples collected, but **42 results remain pending** due to a delay delivery in more than 5 days.
- **South Kivu:** Installation of an INRB mobile laboratory in Miti-Murhesa to accelerate diagnosis.

4.4 Infection Prevention and Control (IPC)

- **Ituri:** assessments in Bunia Cité, decontamination at HGR Bunia, 4 EDS in Bunia, 5 EDS in Nyankunde, 6 EDS in Rwampara, monitoring of the 11 service providers.
- **North Kivu:** Alarming average IPC score of **40 %**. Training of 40 service providers at Virunga General Referral Hospital and briefing of 126 staff members at Heal Africa.
- **South Kivu:** Provision of supplies (chlorine, thermal scanners) to 40 priority establishments through the support of The NGO AFPDE. Evaluation of 35 establishments in 8 health zones.

4.5 Holistic care

Table 4. Patient movement within healthcare facilities as of June 2, 2026

Indicator	Ituri	North Kivu	South Kivu	Together
Patients in bed (Day -1)	191	8	5	204
Total admissions (24h)	28	1	1	30
Exits (24h)				
Deceased (Suspects/Confessions)	8	1	0	9
No cases / Cured		0	0	8
Escapees (Suspects/Confessions)	8 11	0	0	11
Total outflows	27	1	0	28
Patients in isolation (End of Day),	192	8	6	206
including confirmed cases	40	5	2	47
(NC+AC) and suspected cases	152	3	4	159

4.6 CREC and Community Engagement

- **North Kivu:** 22,580 people reached through awareness campaigns. **511 critical reports of resistance. community-based**, reflecting a structural distrust.
- **South Kivu:** Awareness-raising of 399 schoolchildren (Miti-Murhesa and Ibanda health zones) and briefing of 40 RECOs Kavumu.
- **Rumors identified:** Persistent belief in traditional herbal treatments and mistrust of vaccines (especially polio).

4.7 Logistics and Security

- **Official handover of three (3) vehicles (Government allocation) by the Governor of Ituri province to the CDPS Ituri** ; two of these vehicles were immediately allocated to the health zones of Rwampara and Nyakunde.
- Delivery of infection prevention and control (IPC) supplies to the health zones of Mambasa, Mandima, Niania and Mongbwalu.
- Delivery of medicines and IPC supplies with the support of the UG-PDSS to the health zone of Rwampara.

4.8. Security

- The security situation remains unstable in the health zones of Nizi and Bambu.
- The security situation in the city of Bunia (Bunia and Rwampara health zones) is relatively calm. However, the presence of armed groups in the territories of Djugu, Irumu and Mambasa continues to limit humanitarian access in several affected or at-risk health zones.
- A serious incident occurred in **Katana (South Kivu)** where the local population attacked the EDS team, leading to the abandonment of the coffin and the handling of the body by the community. In Ituri, an attack at the Bunia cemetery resulted in five injuries among the response teams.

4.9 PSEA (Prevention of Sexual Exploitation and Abuse)

- Signing of the code of conduct on PSEA by community leaders of the sites displaced persons: 49 people, including 23 men and 26 women.
- Awareness-raising of 179 people (4 men, 71 boys, 9 women and 95 girls) at EP Diro, Shun 1 district, in the Mongbwalu health zone.
- Hosted an interactive program on the Tuungane community radio station in Mongbwalu, which recorded 17 live calls and 7 messages.
- Development in progress of the mapping of actors and their positioning in the PSEA inter-agency action plan.
- Briefing of 52 service providers at the Points of Entry (Ituri) and 41 members of the response team in Miti-Murhesa (South Kivu) on the code of good conduct and reporting mechanisms.

5. MAIN CHALLENGES

Challenge	Impact	Action required
No. 1 Reluctance to perform post-mortem sampling (swabs)	Delay in confirmation; underestimation of cases	Intensive community engagement; involvement of religious leaders
2. Insufficient capacity in standardized CTES despite the opening of Rwampara	Potential saturation of existing CTES	Acceleration of the construction of planned CTES
3. Weakness in contact tracing (overall rate 45.5%, target 95%)	Interruption of compromised transmission chains	Training, deployment and motivation of community relays
4. Supply failure in Essential Generic Medicines (MEG) in targeted HGRs	Weakening of overall care	A plea for the allocation of MEG
5. Insufficient PCI equipment (PPE, DLM, chlorine) in North Kivu	Nosocomial risk	Urgent supplies
6. Low escalation of alerts in the 3 provinces	Delay in detection of case	Strengthening community surveillance
7. Insufficient resources financial for all pillars	Slowdown in operations	Resource mobilization (gap of USD 21.5 million)

6. PERSPECTIVES AND PRIORITY ACTIONS (24-72h)

Priority Action			Due date
Responsible pillar			
1	Training of service providers on the use of PCR RADIONE	Laboratory	Day 2
2	Briefing of service providers on Ebola virus disease (EVD) and tuberculosis control (TBC) surveillance in affected and high-risk areas	Monitoring/CREC	Day 3
3	Briefing of community health workers on Ebola virus disease surveillance and SBC (Ituri province)	Monitoring/CREC	Day 3
4	Supervised support from the Director General of INSP in the technical pillars	Coordination	Continuous
5	Catch-up in contact tracing in Ituri (current rate 41.2%) Surveillance		Day 7
6	Strengthening infection prevention and control (IPC) measures and staff training in Nyankunde and other nosocomial areas	PCI	Day 2
7	Deployment of a standardized daily dashboard with KPIs per pillar	Coordination	Day 3
8	Catch-up plan for the 42 samples awaiting analysis in North Kivu	Laboratory	Day 2
9	North Kivu: discharge of the recovered case to Goma (Day 1), response to security incidents (ADF incursion), IPC training at the General Referral Hospital Virunga, CREC extension to 15,000 households		Day 2
10	South Kivu: provision of 2 additional vehicles, installation of an internet kit in Miti-Murhesa, communication credits for IT/ECZS		Day 2

Joint mission in the Mongbwalu Health Zone (INSP-SGI-WHO-UNICEF-WFP) focused on evaluating Ebola response activities



Tripartite meeting between the Ministries of Health and Education Higher and University Education, Scientific Research and Innovation, and the Ministry of National Education and New Citizenship



**FOR ALL
INFORMATION**

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